

PHYSIOTHERAPY CLINICAL CASE STUDY REPORT

CaseGen AI | Generated: 16 May 2026 20:38

CASE INFORMATION

Case Number:	Auto-assigned
Category:	Outdoor
Date:	Today's date
Referring Physician:	Self-referred / Not specified

PATIENT DEMOGRAPHICS

Name:	Rukhsana Bibi
Age:	55 years
Gender:	Female
Marital Status:	Married
Language:	Not Provided
Occupation:	Housewife
Address:	Rawalpindi
Mode of Admission:	Outdoor / OPD

PRESENT COMPLAINT

The patient is a 55-year-old female presenting with a one-month history of right-sided neck pain and stiffness. The pain is described as a dull ache that worsens with specific movements, particularly neck rotation and looking up, and is relieved by rest and the application of heat. The patient reports no recent trauma or sudden jerking movements that may have precipitated the onset of symptoms. The primary concern expressed by the patient is the persistent nature of the pain and its impact on daily activities, necessitating a comprehensive assessment to determine the underlying cause and appropriate management strategy.

HISTORY OF PRESENTING COMPLAINT

The patient's symptoms began approximately one month ago, with a gradual onset of right-sided neck pain and stiffness. The pain is exacerbated by movements that involve rotation of the neck and looking upwards, with the patient reporting a Numeric Pain Rating Scale (NPRS) score of 6 out of 10 during these activities. In contrast, rest and the application of heat provide relief, suggesting a potential musculoskeletal origin for the pain. The patient denies any history of trauma or sudden jerking movements that may have contributed to the development of symptoms. The patient's past medical history is notable for mild hypertension, which is managed conservatively. The patient's symptoms have been persistent, prompting her to seek medical attention for further evaluation and treatment.

PAIN PROFILE

Location:	Right-sided neck
Onset:	Gradual
Duration:	1 month
Nature:	Dull aching pain with occasional sharp exacerbations
Radiation:	No radiation to upper limbs
NPRS Score:	6/10 at rest, 8/10 on movement

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6/10 at rest, 8/10 on

Aggravating Factors

- Neck rotation
- Looking up

Relieving Factors

- Rest
- Heat pack application

PAST HISTORY

Medical History:	Mild hypertension, managed conservatively
Surgical History:	No previous surgeries reported
Medications:	Not Provided
Family History:	Not Provided
Socioeconomic:	Not Provided
Pre-morbid Status:	Independent in all activities of daily living prior to the current episode
Growth & Development:	Not applicable / within normal limits

PATIENT'S PRIORITIES & FUNCTIONAL GOALS

- Reduce neck pain to allow comfortable sleep
- Improve range of motion in the neck to facilitate daily activities
- Return to normal household duties without exacerbating pain

GENERAL HEALTH STATUS

Vitals

Blood Pressure:	Normal / Within limits
Pulse:	Normal / Within limits
Respiratory Rate:	Normal / Within limits
Temperature:	Afebrile
O2 Saturation:	Normal / Within limits
Weight:	Not Provided
Height:	Not Provided
BMI:	Not Provided

Clinical Examination

Lymph Nodes:	Not palpable
Jaundice:	Absent
Pallor:	Absent
Cyanosis:	Absent
Edema:	Absent

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Clubbing:	Absent
Overall Impression:	The patient appears to be in a stable condition, with no overt signs of distress or systemic illness.

GENERAL HEALTH CONDITION

Malaise:	None reported
Fatigue:	None reported
Fever:	Afebrile
Weight Changes:	None reported
Sleep:	Not reported
Cognition:	Intact
Mood / Affect:	Cooperative and oriented

SYSTEMS REVIEW

Cardiovascular:	No complaints reported
Pulmonary:	No complaints reported
Gastrointestinal:	No complaints reported
Urinary:	No complaints reported
Integumentary:	Skin intact, no wounds or lesions noted
Endocrine:	No complaints reported
Reproductive:	Not assessed / Not applicable

MUSCULOSKELETAL SYSTEM

Gross Symmetry:	Normal
Posture:	Slightly altered due to neck pain, with a tendency to avoid full extension and rotation
Deformity:	None observed
Swelling:	None observed
Tenderness:	Tenderness noted over the right lower cervical paraspinal muscles
Muscle Wasting:	None observed
Muscle Spasm:	Muscle spasm noted in the right cervical region
Cervical ROM:	Restricted in extension and right rotation due to pain
Lumbar ROM:	Normal
Upper Limb ROM:	Normal
Lower Limb ROM:	Normal
Muscle Strength:	Normal throughout

Special Tests

- No specific tests performed during this assessment

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NEUROLOGICAL ASSESSMENT

Consciousness:	Conscious and oriented to time, place, and person
Numbness / Tingling:	None reported
Reflexes:	Normal
Muscle Tone:	Normal
Coordination:	Normal
Gait:	Normal
Cranial Nerves:	Not assessed / Intact
Upper Motor Neuron:	No signs of UMN lesion
Lower Motor Neuron:	No signs of LMN lesion

FUNCTIONAL STATUS

ADL Status:	Independent, with some difficulty due to neck pain
Mobility:	Independent
Work Status:	Modified duties due to neck pain
Recreational:	Limitations in recreational activities that involve neck movement
Assistive Devices:	None currently in use

Functional Goals

- Improve neck range of motion
- Reduce pain during daily activities
- Return to full household duties

INVESTIGATIONS & IMAGING

Imaging:	None provided / Not available
Lab Tests:	None provided / Not available
EMG / NCS:	Not assessed
Other:	None

CLINICAL IMPRESSION & DIAGNOSIS

Cervical Spondylosis with muscle spasm, likely contributing to the patient's symptoms of neck pain and stiffness.

HYPOTHESIS / PROBLEM STATEMENT

The patient's presentation of right-sided neck pain and stiffness, exacerbated by specific movements and relieved by rest and heat, suggests a musculoskeletal origin, likely related to cervical spondylosis with associated muscle spasm. The patient's symptoms have resulted in functional limitations, including difficulty with daily activities and recreational pursuits. A comprehensive treatment plan, incorporating heat therapy, gentle cervical mobilizations, and neck isometric exercises, aims to reduce pain, improve range of motion, and restore functional ability. The patient's mild hypertension, managed conservatively, does not appear to be directly related to the current symptoms but will be considered in the overall management strategy.

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TREATMENT PLAN

Short-Term Goals (0 - 2 Weeks)

- Reduce neck pain by 20% within 2 weeks
- Improve cervical range of motion by 10 degrees within 2 weeks

Long-Term Goals (3 - 6 Weeks)

- Achieve pain-free neck movement within 6 weeks
- Return to full household duties without exacerbating pain within 6 weeks

Physiotherapy Interventions

- Heat therapy: 15 minutes, twice daily, to the cervical region
- Gentle cervical mobilizations: 3 sets, 10 repetitions, 3 times a week, to improve range of motion and reduce stiffness

Home Exercise Program

- Chin tucks: 3 sets x 10 repetitions, hold 5 seconds, 3 times a day, to strengthen neck muscles
- Cervical rotations: 3 sets x 10 repetitions, 3 times a day, to improve range of motion

Patient Education

- Ergonomic workstation setup for neck posture
- Proper lifting techniques to avoid exacerbating neck pain

Referrals: None at this time

Follow-up: Review in 1 week. Full reassessment at 3 weeks.

Prognosis: The patient is expected to make significant improvements in neck pain and range of motion with a comprehensive treatment plan, allowing for a return to full functional activities within 6 weeks.

Date: 16 May 2026

Physiotherapist Signature

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